

PUBLICATION INFORMATION

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THE FAIRBANKS HANDBOOK

Institutional Transformation Handbook

Version 1.0.1

Parent: FairBanks Blueprint — Founding Edition Version 1.0.1 (Master Freeze)

2027–2032 Strategic Horizon

Your Health, Our Mission.

Published by

FAIRBANKS MEDICAL CENTRE LIMITED
Plot 1423 and 1425, Tirupati Road, Fairbanks Medical Centre
Lugogo, Kololo IV, Kampala Central Division
Kampala, Uganda

Institutional particulars (URA Notice DT-1112)

- TIN: 1053370026
- Legal / Business Name: FAIRBANKS MEDICAL CENTRE LIMITED
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Document architecture

- Blueprint = Strategy
- Handbook = Operating System (this book)
- Management Toolkit = Working templates and forms (companion file)
- SOPs = Detailed procedures (separate manuals)
- Handbook v1.0.0 = Compact executive operating guide (kept)

Strategy lives in the Blueprint. Execution lives in this Handbook.

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PART I — UNDERSTANDING THE FAIRBANKS WAY

1. Purpose and Use of the Handbook

This Handbook is FairBanks' institutional operating system for the 2027–2032 strategic horizon. It shows managers, supervisors and teams who does what, when it happens, what evidence is required, and what to do when performance is off-track.

What it is / is not

- Is: operating system for transformation management
- Is not: a second Blueprint; not SOPs; not a marketing brochure
- Does not replace Manifesto, Declaration or Covenant
- Does not invent parallel frameworks

Who uses it / when

- Board — Gate decisions, oversight, stewardship
- MD / Executive — annual plans, stage control
- Directors / Heads — departmental plans, KPIs, corrective action
- Managers / teams — weekly and monthly delivery
- Use at planning, monthly reviews, quarterly reviews, Gates, onboarding, and when terminology drifts

2. Institutional Transformation at a Glance

- Identity: Family & Community Health Social Enterprise
- Horizon: 2027–2032
- Seven priorities: Clinical Excellence; Family & Community Health; Digital; Research & Innovation; Partnerships; Sustainable Financing; Institutional Excellence
- Six frameworks: FIHE, FITF, F7TM, FMM, FCHIP, CHIP (+ supporting instruments via FIMS)

Locked hierarchy

Purpose → Vision/Mission → Seven Strategic Priorities → Six Core Frameworks → Supporting Instruments → FIMS → Five-Stage Transformation Roadmap → Governance/Financing/Partnerships/MEL/Risk/Investment → Manifesto/Declaration/Covenant

How the system works

- BOARD — sets direction, approves Gates, protects stewardship
- EXECUTIVE LEADERSHIP — annual plans and stage ownership
- FIMS — integrates frameworks and instruments
- STRATEGIC PRIORITIES — focus effort
- DEPARTMENTS / PROGRAMMES — deliver services and projects
- PROJECTS & DAILY OPERATIONS — care, outreach, finance, admin
- MEL + LEARNING — measure, review, improve
- TRANSFORMATION GATE — evidence-based progression
- NEXT STAGE — capability-driven advance

PART II — THE FAIRBANKS OPERATING SYSTEM

3. Operating Model

Reporting without correction is not transformation.

Step	Question
Strategy	What are we trying to achieve? (Blueprint)
Annual Plan	What must happen this year?
Department Plan	What must my department deliver?
Individual Responsibilities	What am I accountable for?
KPI	How will we know?
Review	Are we on track?
Corrective Action	What must change?
Learning	What did we learn?

4. Roles and Accountability (RACI)

R = Responsible · A = Accountable · C = Consulted · I = Informed. Clarify if two people are marked A.

Activity	Board	MD	Director	Manager	Staff
Strategic planning	A	R	C	C	I
Annual work plans	I	A	R	R	C
KPI monitoring	I	A	R	R	C
Risk management	A	R	R	R	C
Transformation Gate	A	R	C	I	I
Clinical quality & safety	A	R	R	R	R
Community programmes	I	A	R	R	R
Partnership agreements	A	R	R	C	I
Budget & cash control	A	R	R	C	I
FCHIP / data quality	I	A	R	R	R
MEL dashboards	I	A	R	R	C
Covenant behaviours	A	R	R	R	R

PART III — FIVE-STAGE TRANSFORMATION PLAYBOOK

5. Journey Overview and Principles

The roadmap is time-bound, but transformation is capability-driven.

Period	Stage	Emphasis
2027	Foundation	Build before scaling
2028	Integration	One system, not silos
2029	Innovation	Pilot with evidence
2030–2031	Leadership	Depth, partnerships, succession
2032	Sustainability	Stewardship and endurance

The Leadership stage spans 2030–2031, reflecting the time required to consolidate institutional influence, partnerships, and leadership capacity.

Implementation principles

- Build Strong Foundations Before Expansion
- Strengthen Systems Before Scaling Services
- Promote Partnerships Before Duplication
- Measure Progress Before Accelerating Growth
- Pursue Sustainability Before Replication

6. 2027 — Foundation

Goal

Establish governance, systems, people, finance, digital and operational foundations.

Exit test

Foundations can support Integration without unsafe scale-up.

Actions

- Governance: policies, committees, reporting lines
- People: structure, JDs, performance expectations, orientation
- Finance: budget controls, cash-flow, timely management accounts
- Clinical: SOPs, safety monitoring, referral pathways
- Digital: FCHIP foundation (phased), data governance, information security

KPI examples

- % core policies approved
- % staff with current JDs
- Monthly financial reports on time
- Safety incidents reviewed on time
- % priority SOPs implemented
- Data completeness rate

Gate evidence (minimum)

- ✓ Board/management minutes
- ✓ Approved policies register

- ✓ Staff structure and JD pack
- ✓ Approved budget
- ✓ KPI baseline
- ✓ Risk register
- ✓ SOP register

7. 2028 — Integration

Goal

Integrate clinical, community, digital and workforce pathways.

Exit test

Patients and teams experience one system, not siloed units.

Actions

- Map journey: Registration → Triage → Consultation → Lab → Pharmacy → Payment → Follow-up
- Find delay, duplication and fragmentation
- Document referrals; close community–facility loops; record follow-ups
- Hold multidisciplinary reviews; improve handoff documentation

KPI examples

- Referral feedback loop completion
- Waiting times at critical steps
- Multidisciplinary reviews held
- Patient experience themes
- Documentation completeness

Gate evidence (minimum)

- ✓ Patient journey map
- ✓ Integration checklist results
- ✓ Referral SOP
- ✓ Closed-loop referral samples
- ✓ Quality/risk reports

8. 2029 — Innovation

Goal

Learn, pilot and improve with responsible digital intelligence.

Exit test

Innovation improves outcomes or efficiency with documented learning.

Actions

- Method: Problem → Idea → Pilot → Evidence → Decision → Scale
- No scale without evidence
- FCHIP phased backlog under clinical governance
- Capture lessons; feed practice and training

KPI examples

- Pilots with completed evaluation
- % with Stop/Modify/Scale decision
- Documented gains
- FMM improvement on priority dimensions

Gate evidence (minimum)

- ✓ Innovation canvases
- ✓ Pilot evaluations
- ✓ Scale/stop decisions
- ✓ FCHIP backlog
- ✓ Lessons learned log

9. 2030–2031 — Leadership

Goal

Strengthen FairBanks as a trusted institutional leader and partner.

Exit test

FairBanks can lead collaboratively without founder-only dependence.

Actions

- Second-line leaders ready; documented delegation; succession notes
- Institutional (not personality-based) partnerships
- Credible external representation; documented institutional memory

KPI examples

- Leadership coverage for critical roles
- % partnerships with owners
- Succession docs for key posts
- External engagement quality

Gate evidence (minimum)

- ✓ Leadership readiness assessment
- ✓ Succession notes
- ✓ Partnership tracker
- ✓ Knowledge register
- ✓ Engagement log

10. 2032 — Sustainability

Goal

Lock stewardship, resilience and lasting community value.

Exit test

The institution can endure beyond one planning cycle.

Actions

- Stewardship test before major spend (finance, operations, safety, leadership, community value, endurance)
- Confirm resilience; reaffirm Covenant; prepare next horizon

KPI examples

- Sustainability/resilience indicators
- % major investments passing stewardship test
- Risk residual within appetite
- Covenant reaffirmation done

Gate evidence (minimum)

- ✓ Investment prioritisation records
- ✓ Sustainability scorecard
- ✓ Risk register
- ✓ Covenant minute
- ✓ Next-horizon briefing

11. Transformation Gates

Do not advance only because a calendar year ended. Progression depends on capability evidence.

Gate evidence pack

1. Stage scorecard
2. Deliverables tracker
3. Financial position
4. Workforce readiness
5. Risk register
6. Patient safety report
7. MEL dashboard
8. Partnership status
9. Lessons learned
10. Management recommendation

Board decision options

- PROCEED
- PROCEED WITH CONDITIONS
- HOLD & REMEDIATE
- REDESIGN SCOPE

Interpretation criteria (proposed for Board ratification)

The bands below are a proposed institutional standard for Gate interpretation. They should be confirmed or amended by FairBanks leadership before being treated as binding thresholds.

Rating	Meaning (proposed)	Typical Gate leaning
Green	≥90% critical deliverables complete; no unresolved critical patient-safety issue; finance and workforce capacity confirmed for next stage	Proceed
Amber	70–89% critical deliverables complete;	Proceed with conditions or Hold

	manageable risks; documented recovery plan with owners and dates	
Red	<70% critical deliverables; material safety/financial/governance concern; insufficient capacity for next stage	Hold & remediate or Redesign scope

Critical patient-safety, safeguarding, or legal non-compliance overrides percentage scores and defaults to Hold / Redesign until resolved.

PART IV — MANAGEMENT RHYTHM

12. Annual Planning

Every department completes an Annual Department Work Plan (Toolkit Tool 1) aligned to a Blueprint priority and stage.

- Strategic Priority · Blueprint Anchor · Annual Objective · Key Activities
- Responsible Person · Budget · KPI · Target · Timeline · Risks · Evidence

13. Monthly Performance Review

1. What did we commit to?
2. What was achieved?
3. What was not achieved?
4. Why?
5. What risks emerged?
6. What corrective action is required?
7. Who owns the action?
8. When will it be completed?
9. What evidence will demonstrate completion?

14. Quarterly Transformation Review

RAG rate each domain: Green On Track · Amber At Risk · Red Off Track.

- Clinical
- Community
- Finance
- People
- Digital
- Partnerships
- Research
- Risk
- MEL

15. Annual Institutional Review

- Stage review against Blueprint Chapter 10
- Policy/SOP refresh
- Covenant reaffirmation
- Next-year plan within 2027–2032
- Board Gate decision if at a stage boundary

PART V — PERFORMANCE AND LEARNING

16. MEL and Dashboards

- Small vital indicator set per department
- Clear owner, source, frequency, corrective path
- Review monthly; deep-dive quarterly
- Escalate persistent red ratings

17. FMM Self-Assessment

Use FairBanks Maturity Model quarterly on priority dimensions. Scores guide capability building — not vanity.

18. Risk Management

Review risks monthly (management) and quarterly (Board). Use Toolkit Tool 5.

19. Learning and Knowledge Management

- Capture lessons after projects, incidents and Gates
- Store where successors can find them
- Convert repeated lessons into SOP/training updates
- Never scale innovation without a recorded learning note

19A. Evidence Standards

Evidence used for reviews and Gates should be:

- Relevant — linked to the objective
- Verifiable — someone else can confirm it
- Current — reflects the reporting period
- Traceable — source is identifiable
- Actionable — supports a decision

“Patient experience improved” is not sufficient evidence.

Prefer statements such as: patient feedback for the reporting period, using the approved tool, with documented corrective actions for the largest complaint themes. Numbers and tools must come from FairBanks' approved MEL instruments — do not invent figures in the Handbook.

PART VI — FUNCTIONAL IMPLEMENTATION

20. Clinical Services & FIHE

Core actions

- FIHE continuity
- Quality & safety huddles
- Referral pathways
- Reduce fragmentation

Example KPIs

- Safety incidents closed on time
- Documentation completeness
- Referral loop closure

21. Nursing

Core actions

- Patient experience & dignity
- Handovers
- Community linkage
- Early safety escalation

Example KPIs

- Handover compliance
- Complaint themes
- Escalation timeliness

22. Laboratory

Core actions

- Quality & TAT
- Equipment uptime
- Reliable result flow

Example KPIs

- TAT vs target
- QC performance
- Critical result notification

23. Pharmacy

Core actions

- Stock availability
- Rational use
- Counselling
- Revenue integrity without harming care

Example KPIs

- Stock-out days
- Dispensing errors
- Counselling audit

24. Finance & Sustainability

Core actions

- Budget & cash discipline
- Timely accounts
- Stewardship test before major spend

Example KPIs

- Reports on time
- Budget variance
- Cash days cover

25. HR / Corporate Services

Core actions

- Staffing
- Performance & training
- Culture
- Support services

Example KPIs

- Critical posts filled
- Appraisal completion
- Priority training

26. Community Health, CHIP & Outreach

Core actions

- Need-based outreach
- CHW/VHT support
- Referral linkage
- CHIP phased/designed until evidenced

Example KPIs

- Outreach completed
- Referral completion
- CHIP pilot integrity

27. FCHIP & Digital

Core actions

- Phased backlog

- Privacy & security
- Workflow-first tools
- Use-case owners

Example KPIs

- Data completeness
- Downtime
- Use-case delivery

28. Partnerships

Core actions

- Pipeline & agreements
- Owners & deliverables
- Collaborate before duplicating

Example KPIs

- Active agreements
- Deliverable completion
- Issues >30 days

29. MEL

Core actions

- Institutional dashboard
- Support usable KPIs
- Gate packs
- Learning reviews

Example KPIs

- Dashboard freshness
- Gate pack completeness
- Learning notes filed

30. Research & Innovation

Core actions

- Innovation Canvas
- Ethics/governance
- Stop/Modify/Scale with evidence

Example KPIs

- Pilots evaluated
- Decisions recorded
- Practice changes

31. People & Leadership Development

Core actions

- Reduce founder-only dependence
- Coach second line
- Succession
- Institutional memory

Example KPIs

- Leadership coverage
- Succession docs
- Delegation evidence

PART VII — CULTURE AND STEWARDSHIP

32. FairBanks Culture in Daily Decisions

Before acting, ask:

- Does this improve patient care?
- Does this strengthen the family?
- Does this benefit the community?
- Does this protect institutional sustainability?
- Does this create useful knowledge?
- Does this strengthen trust?
- Does this honour the Covenant?

33. Covenant in Practice

- Reaffirm annually at Board
- Use in induction and appraisals
- Use in difficult trade-offs

34. Leadership and Succession

- Map critical roles and deputies
- Develop second-line leaders
- Document and share trapped knowledge
- Delegate real authority

35. Institutional Stewardship

Every major investment or expansion must pass the Sustainability Playbook stewardship questions.

PART VIII — TOOLKIT POINTER AND QUICK REFERENCE

36. Management Toolkit (companion)

Blank forms, registers and scorecards are maintained in the separate companion: FairBanks Management Toolkit v1.0.0. The Handbook stays compact; managers print and reuse Toolkit forms.

Tool	Name
1	Annual Department Work Plan
2	Monthly Performance Dashboard
3	Quarterly Transformation Scorecard (RAG)
4	Transformation Gate Evidence Checklist
5	Risk Register
6	Partnership Tracker
7	Innovation Proposal / Canvas
8	Project Status Report
9	Lessons Learned Template
10	Corrective Action Register
11	Staff Transformation Self-Assessment
12	Department Head Readiness Assessment
13	Board Gate Decision Form

37–38. First 90 Days

Day 1–30 — Understand

- Read Blueprint Executive Summary, At a Glance, Chapter 10
- Read Handbook Parts I–III
- Clarify role and stage

Day 31–60 — Align

- Confirm objectives and KPIs
- Log top risks
- Agree evidence
- Join monthly rhythm

Day 61–90 — Execute

- Deliver 3–5 milestones
- Close one corrective action
- File one lesson learned
- Know next Gate/quarterly date

39. Escalation rules

- Patient safety — escalate immediately via clinical chain
- Red KPI for two months — escalate to Director/MD
- Material financial/partnership risk — escalate to MD/Board as required
- Terminology drift / parallel frameworks — stop; return to Blueprint/Handbook

40. Institutional Language & Terminology

- Say “2027–2032 strategic horizon” (not “five-year strategic period”)

- Say “five-stage transformation journey”
- Keep stages: 2027 Foundation → 2028 Integration → 2029 Innovation → 2030–2031 Leadership → 2032 Sustainability
- Treat FCHIP/CHIP as phased/designed unless evidence shows otherwise
- Use URA address and TIN 1053370026 for formal particulars
- Catchment names (e.g. Kyebando) are outreach geography — not headquarters address

41. Success picture for tracking

By the end of the 2027–2032 strategic horizon, FairBanks aims to be recognised for:

- Making significant progress toward becoming Uganda's leading Family & Community Health Social Enterprise
- Trusted for integrated healthcare services
- Recognised for excellence in community health and preventive care
- Recognised as a leader in digital health and community health intelligence
- Recognised as a respected research and innovation partner
- Recognised as a preferred partner for government, universities, insurers, and development agencies
- Financially sustainable, with measurable social impact

42. Closing rule

Don't reinvent the strategy. Don't rush the transformation. Don't compromise the values.

- Does the Blueprint already decide this? If yes, implement — do not reopen.
- Are we ready for the next stage by capability, not by calendar? If no, hold the Gate.
- Does this action honour the Covenant and “Your Health, Our Mission”? If no, stop.

End of FairBanks Institutional Transformation Handbook v1.0.1 — MASTER FREEZE

Companion: FairBanks Management Toolkit v1.0.0 · Parent: Blueprint v1.0.1